Documentation of Evidence-Based Advocacy on Anti-GBV National Policy and Integration into the GEWE Policy



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ACRONYMS AND ABBREVIATIONS

ACRONYMS	
GBV	Gender-Based Violence
GBV-CE	Gender-Based Violence Campaign Ethiopia
HTPs	Harmful Traditional Practices
MoWSA	Ministry of Women and Social Affairs
EWLA	Ethiopian Women Lawyers Association
EWRA	Ethiopian Women Rights Advocates
FDRE	Federal Democratic Republic of Ethiopia
FGM/C	Female Genital Mutilation/Cutting
GBV	Gender-Based Violence
CSOs	Civil Society Organizations
EWLA	Ethiopian Women Lawyers Association
EWRA	Ethiopian Women Rights Advocates
FGM/C	Female Genital Mutilation/Cutting
NEWA	Network of Ethiopian Women's Associations

EXECUTIVE SUMMARY

This report provides a comprehensive documentation and analysis of the evidence-based advocacy journey that led to the development and subsequent integration of a dedicated Anti-Gender-Based Violence (GBV) policy into Ethiopia's revised national Gender Equality and Women's Empowerment (GEWE) Policy. This integration is framed not as a compromise, but as a significant strategic achievement and a testament to the effectiveness of sustained, multi-stakeholder advocacy.

The advocacy process was characterized by a powerful coalition led by the Ministry of Women and Social Affairs (MoWSA) in close partnership with women's rights organizations (WROs), civil society organizations (CSOs), and UN agencies. A cornerstone of the strategy was its rigorous evidence-based approach, which utilized data from national surveys like the Ethiopia Demographic and Health Survey (EDHS), court monitoring reports, legal gap analyses, and survivor testimonies. This evidence was critical in building an irrefutable case for reform, highlighting gaps in the existing legal framework concerning issues like marital rape, acid attacks, and digital violence.

Key strategies employed included strategic coalition-building, high-level policy dialogues and technical working groups, public awareness campaigns leveraging initiatives like the 16 Days of Activism, and persistent lobbying of government institutions. These concerted efforts culminated in the government's acceptance of the comprehensive Anti-GBV policy content. The decision to integrate this content into the overarching GEWE Policy is a key outcome. It successfully embeds GBV prevention and response at the heart of the national gender equality agenda, ensuring it receives sustained attention and is aligned with broader efforts to empower women.

Despite this success, the process faced considerable challenges, including bureaucratic delays, fluctuating political will, limited engagement from some key government ministries, funding constraints, and deep-rooted socio-cultural norms that normalize GBV. The underutilization of digital advocacy tools also represented a missed opportunity for broader outreach.

The report concludes that the integration of the Anti-GBV policy is a positive and foundational outcome that reflects both advocacy success and government commitment. It provides a durable platform for action. The report recommends a strategic shift in future advocacy: first, to focus on ensuring the robust implementation, monitoring, and resourcing of the integrated policy; and second, to use the evidence and experience generated from this implementation to build a compelling case for future advocacy, which may include a push for a standalone Anti-GBV law. The journey underscores the critical importance of collaborative, adaptive, and evidence-informed advocacy in achieving transformative policy change in Ethiopia.



SECTION 1.
INTRODUCTION

Background and Context

Gender-based violence (GBV) remains one of the most severe and pervasive human rights violations in Ethiopia, a manifestation of deep-seated gender inequalities and patriarchal power structures. Despite the progressive provisions enshrined in the FDRE Constitution (1995) and the existence of sectoral laws, the national response to GBV has been hampered by a fragmented legal framework and significant protection gaps. The 2016 Ethiopia Demographic and Health Survey (EDHS) starkly illustrates the scale of the problem, indicating that nearly a quarter of women have experienced physical violence and a tenth have experienced sexual violence, with a staggering 66% of survivors not reporting their experiences (Central Statistical Agency [CSA] & ICF, 2016).

In response to this critical challenge, a broad coalition of government, civil society, and international partners initiated a concerted advocacy campaign for a standalone, comprehensive Anti-GBV policy. This report documents that journey, which achieved a pivotal outcome: the integration of the core components of the proposed Anti-GBV policy into the revised national Gender Equality and Women's Empowerment (GEWE) Policy. This integration is a clear indicator of advocacy success, demonstrating the government's commitment to addressing GBV and strategically mainstreaming the issue within Ethiopia's primary framework for achieving gender equality.

Objectives of the Assignment

General Objective:

To capture and analyze the process, tools, and lessons from the evidence-based advocacy led by women's rights organizations and their partners that contributed to the development and integration of the Anti-GBV policy into the GEWE Policy.

Specific Objectives:

- To document the advocacy journey, including key milestones, strategies, and actors involved in the development and promotion of the Anti-GBV policy.
- To examine and positively frame the integration of the Anti-GBV policy into the broader GEWE Policy as a key achievement of the advocacy efforts.
- To identify and analyze the types of evidence used to support the formulation, endorsement, and integration of the policy framework.
- To explore the specific roles, contributions, and dynamics of WROs, CSOs, networks, and other actors in shaping public discourse and influencing government commitment.
- To highlight the enabling and limiting factors that influenced the success and direction of the advocacy efforts.
- To extract key lessons learned and best practices that can inform ongoing and future gender-responsive legal and policy advocacy in Ethiopia.

Scope

The scope of the assignment encompasses a comprehensive evaluation of the Anti-GBV policy advocacy process in Ethiopia. It covers the period from the initial mobilization and agenda-setting through the final decision to integrate the policy into the GEWE Policy. The analysis includes documentation of the advocacy strategies and tools employed, an examination of the multi-stakeholder collaboration, an assessment of the types and use of evidence, and an evaluation of the factors that facilitated or hindered the process. The scope focuses on national-level advocacy while acknowledging regional linkages and constraints.



Methodology

The study employed a qualitative research design to construct a detailed understanding of the advocacy process. The primary methods were:

- **Document Analysis**: A systematic review of both printed and electronic materials was conducted. This included policy drafts, project activity reports from CSOs and UN agencies, civil society opinion reports on the draft policy, relevant national strategies, and international literature on GBV and policy advocacy.
- **Key Informant Interviews (KIIs):** Semi-structured interviews were conducted with 10 individuals who were directly involved in the advocacy process. Participants were selected purposively and included representatives from MoWSA, UN agencies (UN Women, UNFPA), and leading WROs/CSOs (e.g., NEWA, EWLA, EWRA)
- **Questionnaires**: Fifteen semi-structured questionnaires were administered to a wider group of stakeholders to gather supplementary data on their participation, perceptions of collaboration, and evaluation of the outcomes.

The collected data was transcribed, organized, and analyzed using thematic analysis. Key themes were identified iteratively, including collaboration dynamics, evidence use, strategy effectiveness, and challenges.

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Limitations

The study was conducted under constraints of time and financial resources, which limited the geographic scope of primary data collection largely to Addis Ababa. As the policy integration is a recent development, the availability of published literature and retrospective analyses was limited. Furthermore, the study relied on the recall and perspectives of participants, which may be subject to bias. The evaluation of the advocacy's impact is also limited to the policy integration itself, as the implementation phase and its effects are yet to be fully realized.

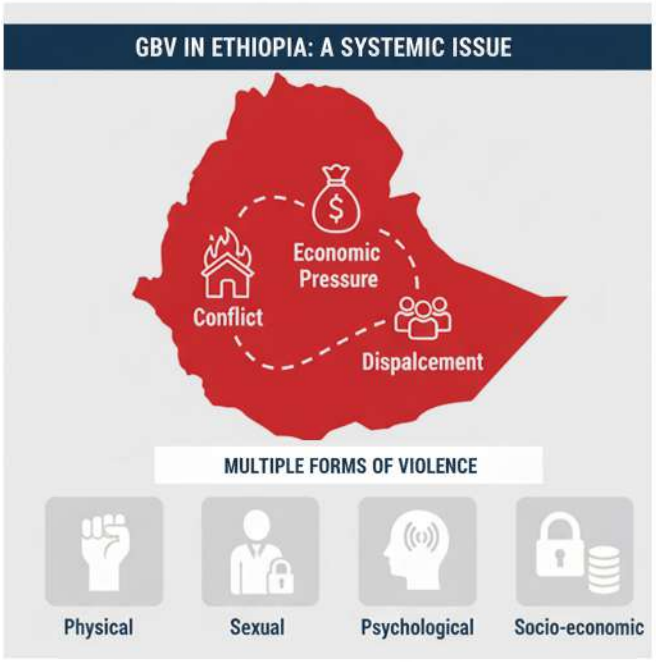


SECTION 2.

LITERATURE REVIEW

2.1 The Pervasiveness of GBV and the National Context

Gender-Based Violence in Ethiopia is not a series of isolated incidents but a widespread systemic issue. The EDHS 2016 data provides a foundational understanding, but more recent reports and studies indicate that the situation may be exacerbated by conflict, displacement, and economic pressures. GBV manifests in multiple forms, including physical, sexual, psychological, and socio-economic violence. The World Health Organization (2012) defines sexual violence comprehensively, emphasizing that it can be perpetrated "by any person regardless of their relationship to the victim," a definition that underscores the need for laws that protect women in all settings, including the home. Harmful traditional practices like FGM/C and child marriage continue to inflict profound and lasting harm on women and girls.



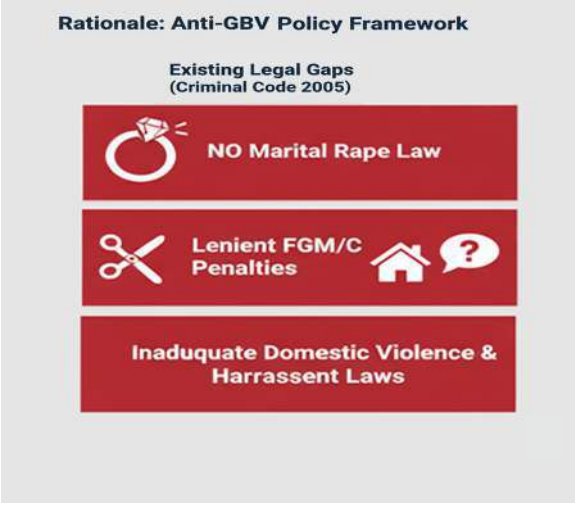


2.2 Evolution of Ethiopia's Policy Framework on Gender and GBV

Ethiopia's policy approach to gender has evolved significantly. The 1993 National Policy on Ethiopian Women was a landmark step, leading to the establishment of the Ministry of Women and Children's Affairs. This was followed by the Growth and Transformation Plan and the subsequent National Strategy and Action Plan on Harmful Traditional Practices (HTPs) against Women and Children in 2013. The GEWE strategy represents the current overarching framework. The integration of the Anti-GBV policy into this evolving architecture marks the latest and one of the most specific steps in strengthening the national response to violence against women.

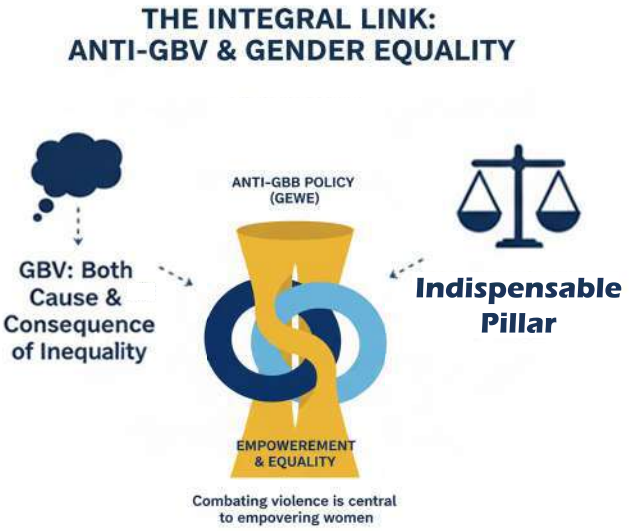
2.3 Rationale for a Dedicated Anti-GBV Policy Framework

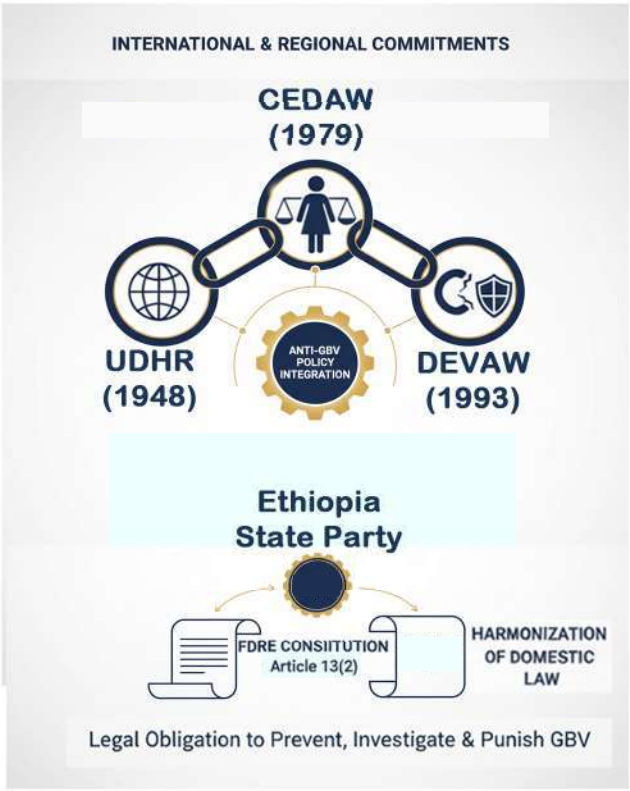
The advocacy for a dedicated policy was driven by clear gaps in the existing legal system. While the Criminal Code (2005) addresses some forms of violence, it fails to criminalize marital rape, provides lenient penalties for FGM/C, and does not adequately define or address domestic violence or sexual harassment in a consistent manner. The draft Anti-GBV policy, developed by MoWSA, was designed to create a coherent, multi-sectoral framework outlining clear responsibilities for prevention, protection, service provision, and accountability, filling the voids left by the sectoral laws.



2.4 The Integral Link between Anti-GBV Policy and Gender Equality

It is widely recognized that GBV is both a cause and a consequence of gender inequality (United Nations, 1993). A society cannot achieve meaningful gender equality while women and girls are subject to violence. Therefore, the integration of the Anti-GBV policy directly into the GEWE strategy is not just administratively convenient but conceptually sound. It ensures that combating violence is recognized as an indispensable pillar of empowering women and achieving equality, rather than a separate or sidelined issue.



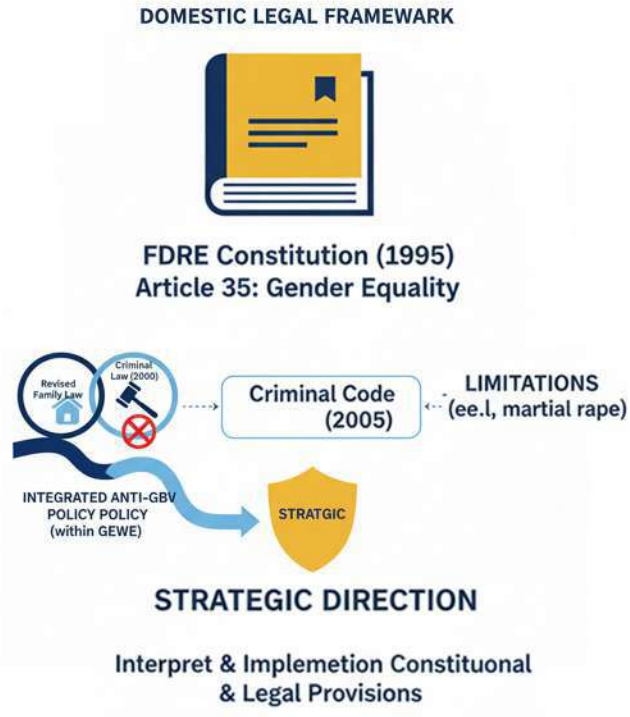


2.5 International and Regional Commitments

Ethiopia is a state party to key international instruments that create a legal obligation to prevent, investigate, and punish acts of GBV. These include the Universal Declaration of Human Rights (UDHR, 1948), the Convention on the Elimination of All Forms of Discrimination against Women (CEDAW, 1979), and the Declaration on the Elimination of Violence against Women (DEVAW, 1993). The integration of the Anti-GBV policy is a concrete step towards harmonizing domestic law with these international commitments, as mandated by Article 13(2) of the FDRE Constitution.

2.6 Domestic Legal Framework

The FDRE Constitution (1995) is the supreme law, and Article 35 provides a strong basis for gender equality and the elimination of HTPs. The Revised Family Law (2000) strengthened women's rights in marriage and property. However, as noted, the Criminal Code (2005) has limitations. The integrated Anti-GBV policy within the GEWE Policy is intended to provide the strategic direction needed to interpret and implement these constitutional and legal provisions more effectively in the context of GBV.



SECTION 3.
DATA ANALYSIS

3.1 Role and Contribution of the Advocacy Alliance

3.1.1 Leadership and Coordination (MoWSA and UN Agencies):

MoWSA played an indispensable role as the government champion, providing the policy process with official legitimacy and coordinating the technical working groups. UN agencies, particularly UN Women, provided critical financial resources, technical expertise on global norms, and convening power. As one interviewee from a UN agency stated, "Our role was to bridge the gap between the global evidence and the national context, and to create spaces for dialogue between CSOs and the government."

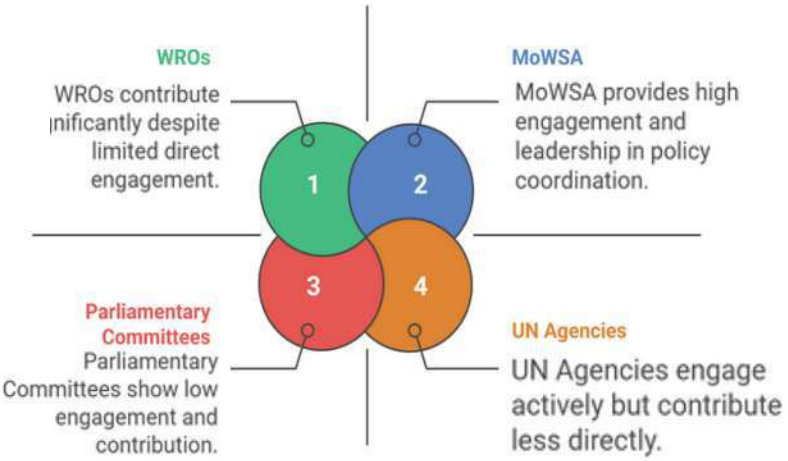
3.1.2 Technical and Grassroots Contributions (WROs and CSOs):

Organizations like EWLA and NEWA were the engine of the advocacy. EWLA's work in providing free legal services generated invaluable court monitoring data and survivor testimonies that exposed the failures of the existing system. NEWA's coordination of the GBV-CE coalition amplified the collective voice of its member organizations. Their grassroots presence allowed them to mobilize community support and bring the realities of GBV to the policy table. A CSO representative noted, "We were the ones who could say to the government, 'This is not just a theory; this is what is happening to women in your courts, in your police stations.

3.1.3 Gaps in Stakeholder Engagement:

A recurring theme in the analysis was the limited direct engagement of key parliamentary committees, other sectoral ministries (e.g., Health, Education, Justice), and mass-based associations. This sometimes created the perception that GBV was solely "MoWSA's issue" or a "women's issue," rather than a cross-cutting national priority, which occasionally slowed broader political buy-in.

Advocacy Alliance Roles and Engagement



3.2 Lessons Learned and Identified Best Practices

3.2.1 The Power of Collaborative Advocacy:

The synergy between government, CSOs, and UN agencies was the single greatest factor in the advocacy's success. It combined insider access with outsider pressure and technical credibility. The coalition was able to present a unified front that was difficult for policymakers to dismiss

Key Factors in Advocacy Success



3.2.2 The Centrality of Evidence-Based Strategies:

The advocacy moved from moral arguments to evidence-driven demands. The use of EDHS data, EWLA's legal case data, and field-based reports transformed GBV from an abstract social ill into a concrete policy problem requiring a legislative solution. This evidence was crucial for countering resistance and building a rational case for change.

3.2.3 Strategic Adaptation and Coalition Management:

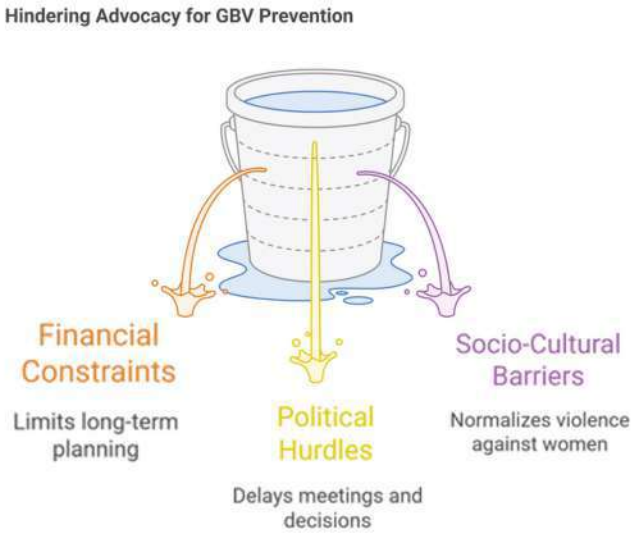
The alliance demonstrated strategic adaptability. When it became clear that a standalone policy faced significant political hurdles, the coalition effectively shifted its tactics to advocate for the substantive integration of the policy content, which was ultimately successful. Managing a diverse coalition required continuous communication and negotiation to align goals and strategies.

3.3 Limitations and Challenges

3.3.1 Financial and Institutional Constraints: Funding for advocacy was often project-based and short-term, limiting long-term planning and the ability to sustain pressure. This also restricted activities to areas that were easier to access, leaving many rural and conflict-affected regions without adequate representation in the advocacy process.

3.3.2 Political and Bureaucratic Hurdles: The government's policy process is inherently slow. Shifting political priorities and bureaucratic inertia often delayed meetings, reviews, and decisions. The advocacy had to consistently work to keep GBV on the agenda amidst competing national prioritial.

3.3.3 Socio-Cultural Barriers: Perhaps the most profound challenge was the deep-seated cultural norms that normalize violence against women. In many communities, practices like FGM and early marriage are deeply embedded, and addressing marital rape is considered an intrusion into private family matters. This required parallel work on social norm change, which is a long-term endeavor.



3.4 Analysis of Collaborative Dynamics

The collaboration between CSOs and the government, particularly MoWSA, was largely project-specific and positive but was not fully institutionalized. There was a degree of dependency on the personal commitment of individual officials within MoWSA.

3.5 Evaluation of the Advocacy Process and Outcome

A comprehensive evaluation of the Anti-GBV policy advocacy process, measured against standard metrics for effective advocacy, reveals a significant and multi-faceted success. While the ultimate goal of a standalone policy was not immediately realized, the strategic integration of its core provisions into the national GEWE Policy represents a profound victory that validates the advocacy strategy and provides a powerful platform for future action.

3.5.1 Substantive Policy Change: A Foundation for Transformation

The most direct measure of advocacy success is observable change in the policy landscape. In this regard, the outcome is unequivocally positive.

- **From Marginal to Mainstream:** The integration of comprehensive Anti-GBV provisions directly into the GEWE Policy is not a minor adjustment but a substantive mainstreaming of the issue. It moves GBV from the periphery of political concern to the very center of the national gender equality agenda. This ensures that GBV prevention and response are no longer an optional or isolated program but a mandatory component of the government's core gender strategy.

- **Adoption of Core Advocacy Demands:** The integrated content reflects the key demands of the advocacy coalition. This includes the adoption of a comprehensive definition of GBV that encompasses emerging forms like digital violence, the establishment of clear frameworks for multi-sectoral coordination (e.g., between health, justice, and social services), and the prioritization of survivor-centered approaches, including the strengthening of one-stop centers. As one MoWSA official noted, "The draft Anti-GBV policy became the primary source document for the GBV chapter of the new Gender Policy. The technical work of the coalition was largely adopted verbatim."

- **Creating a Binding Framework:** Unlike a standalone advocacy report or a civil society charter, the GEWE Policy is a binding national framework. This integration gives the Anti-GBV provisions the force of official government policy, mandating action across federal and regional institutions and creating a basis for accountability.

3.5.2. Demonstrable Government Commitment: A Shift in Stance

The government's decision to integrate the policy is a concrete indicator of high-level commitment, moving beyond rhetorical support to actionable ownership.

Political Ownership: By choosing to embed the policy within its own flagship GEWE strategy, the government has taken ownership of the GBV issue. This is a critical shift from perceiving it as an external demand from civil society to recognizing it as an internal government priority. This ownership is essential for driving implementation and resource allocation.

Strategic Compromise as Success: The integration can be viewed as a strategic compromise that secured the substance of the advocacy demand while allowing the government to manage the form. This outcome demonstrates a level of political pragmatism and sophistication within the advocacy coalition. It achieved the primary objective, a comprehensive national framework, while avoiding a potential political stalemate over the symbolic issue of a standalone law. A UN agency representative reflected, "We shifted the debate from 'if' there should be a policy to 'how' it would be implemented. Integration was the vehicle that secured the 'yes'."

3.5.3. Significant Shift in Public and Political Discourse

The government's decision to integrate the policy is a concrete indicator of high-level commitment, moving beyond rhetorical support to actionable ownership.

- **Elevation on the National Agenda:** Through consistent media engagement, public campaigns like the 16 Days of Activism, and high-level policy dialogues, the coalition successfully framed GBV not as a private or cultural matter, but as a public health crisis, a barrier to development, and a fundamental human rights violation. This is evidenced by increased media coverage of GBV cases and policy discussions, moving the topic from the social sections to the political and news headlines.
- **Creation of a Specialized Policy Vocabulary:** The advocacy introduced and legitimized specific concepts, such as "survivor-centered justice," "multi-sectoral response," "marital rape," and "digital GBV", into the lexicon of policymakers, service providers, and the media. This shared vocabulary is a prerequisite for sophisticated and effective action.
- **Increased Accountability Pressure:** The very public advocacy process has created a cohort of informed stakeholders, including parliamentarians, journalists, and community leaders, who are now aware of the government's policy commitments and are better equipped to demand accountability for their implementation.



Conclusion of a Phase, Launch of the Next

The integration of the Anti-GBV policy is therefore a major achievement that marks the successful conclusion of the policy development and adoption phase of advocacy. It is a testament to the power of evidence-based, collaborative, and strategic activism. However, it also definitively opens the next, and arguably more challenging, phase: **implementation and monitoring.**

The success of this advocacy outcome will now be measured by a new set of metrics: the development of a costed national action plan, the allocation of dedicated budgets, the training of service providers, the establishment of functional coordination mechanisms, and, ultimately, a measurable decrease in the prevalence of GBV and an increase in survivors' access to justice and services. The integrated policy is not the end of the journey, but the critical foundation upon which all future efforts to build an Ethiopia free from gender-based violence will now depend.

SECTION 4. SUMMARY OF FINDINGS

4.1 The Collaboration and Coalition Building Effort

The advocacy's success was fundamentally rooted in a powerful, multi-stakeholder coalition. The partnership between MoWSA, UN agencies, and a core group of WROs/CSOs created a force multiplier effect. Each actor brought unique strengths: MoWSA provided governmental authority and access, UN agencies offered technical and financial resources, and CSOs contributed grassroots legitimacy, evidence, and relentless pressure. The GBV-CE platform was instrumental in coordinating this collective action and maintaining a unified voice.



4.2 Key Barriers to the Advocacy Process

The journey was marked by several significant barriers. Institutional and political challenges, including bureaucratic delays and the lower prioritization of GBV by some government sectors, slowed progress. Financial constraints limited the scope and sustainability of advocacy activities. The most entrenched barrier was socio-cultural resistance, where harmful traditional practices and patriarchal norms in many communities created a hostile environment for GBV reforms. Furthermore, the under-utilization of digital tools limited the campaign's reach and modern engagement potential.



4.3 Consolidated Lessons and Best Practices

The process yielded invaluable lessons. It confirmed that evidence-based advocacy is the most powerful tool for influencing policy. It demonstrated that broad coalitions are essential for amplifying impact and resilience. It highlighted the necessity of strategic adaptability in navigating complex political landscapes. A key best practice that emerged was the effective division of labor within the coalition, allowing different actors to leverage their comparative advantages, from technical drafting to public mobilization.

4.4 Final Evaluation: Integration as a Strategic Success

The integration of the Anti-GBV policy into the GEWE Policy is evaluated as a clear and strategic success for the advocacy movement. It represents a significant milestone that should be celebrated. This outcome achieves the primary goal of securing a government-backed, comprehensive framework for addressing GBV. It strategically positions the issue within the main national vehicle for gender equality, ensuring it receives sustained attention and resources. This provides a solid and legitimate foundation for all future work on GBV in Ethiopia.

5. CONCLUSION AND RECOMMENDASECTIONS



5.1 CONCLUSION

The advocacy journey for the Anti-GBV policy in Ethiopia is a powerful narrative of how sustained, collaborative, and evidence-informed activism can achieve meaningful policy change. The integration of the policy into the national GEWE Policy is a major victory. It is the result of years of strategic effort by a dedicated alliance of government bodies, civil society organizations, and international partners. This achievement reflects a significant commitment from the Ethiopian government to address the scourge of GBV and embeds this commitment within the country's core strategy for gender equality. It provides a durable and authoritative platform from which to launch the critical next phase: turning policy into practice and tangible protection for all women and girls.



5.2 RECOMENDATIONS

1. Publicly Acknowledge and Consolidate the Success: The coalition should formally celebrate the integration as a major achievement. This reinforces the value of the partnership, acknowledges the government's commitment, and builds momentum for the implementation phase.

2. Launch a Focused Advocacy Campaign for Implementation: The immediate advocacy focus must shift from policy development to implementation. This includes lobbying for:

- The swift development and adoption of a costed, time-bound National Action Plan for the integrated GBV components.
- Clear and dedicated budget lines within MoWSA and other relevant ministries (Health, Justice, Education) for GBV prevention and response activities.
- The establishment of a robust, multi-stakeholder monitoring framework to track the implementation of the policy and hold all actors accountable.

3. Generate Evidence for Future Reform: The implementation phase should be used strategically to document what works and what does not. systematically collect data on the strengths and limitations of the integrated approach. This evidence will be crucial for informing future advocacy, whether for strengthening implementation or, in time, making a renewed case for a standalone policy based on practical experience.

4. Broaden and Deepen the Coalition: Intensify efforts to engage a wider array of champions, including members of parliament, religious and traditional leaders, influential media figures, and the private sector. Creating a broader base of ownership is essential for sustaining political will and societal support.

5 Integrate Digital Advocacy Strategies: Future campaigns must incorporate a strong digital component. Utilize social media, online petitions, and digital storytelling to reach younger audiences, disseminate information rapidly, and create new forms of public pressure.



Public Acknowledgment

Formally celebrating integration to reinforce value and commitment



Advocacy Campaign

Lobbying for action plan, budget, and monitoring framework



Evidence Generation

Documenting strengths and limitations for future reforms



Coalition Broadening

Engaging diverse champions to sustain political will



Digital Advocacy

Utilizing digital tools to reach younger audiences



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